TE-08 WMD / Nerve-Agent Exposure

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026

No entry without the right protection. Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime field kit. This protocol does not authorize routine field autoinjector or antidote administration.

Recognition and immediate field flow

Step	Approved action
1. Do not enter	Stage uphill and upwind when possible. Request HazMat, fire, law enforcement, and incident command. Only trained personnel with appropriate monitoring, PPE, and respiratory protection may enter the hazard area.
2. Remove and decontaminate	Protected personnel remove the patient from the source; remove and bag contaminated clothing; brush dry material; and irrigate as directed. Prevent secondary contamination of responders, ambulance, and receiving facility.
3. Airway and ventilation	Suction secretions, provide oxygen and BVM ventilation, and use the applicable seizure pathway. Do not allow antidote retrieval to delay resuscitation.
4. Cholinergic syndrome?	Consider miosis, salivation, lacrimation, sweating, bronchorrhea, bronchospasm, vomiting, diarrhea, urination, bradycardia, fasciculations, weakness, apnea, or seizure after a compatible exposure.
5. Escalate	Call Poison Help (1-800-222-1222) and Medical Control. Request HazMat/mutual aid and early receiving-facility coordination. No CHEMPACK, Mark I, or equivalent field kit is carried by Claiborne ground units.
6. Transport	Transport after indicated decontamination. Communicate agent, route, scene zone, PPE, symptoms, decontamination, airway/ventilatory care, and consultation guidance.

Provider-level actions

Level	Approved actions
EMT	Remain outside the hazard until assigned PPE and training level are appropriate; support removal, decontamination, suction, oxygenation, BVM ventilation, glucose assessment, and rapid transport.
AEMT	All EMT care plus IV/IO access, authorized fluids, reassessment, and preparation for rapid respiratory deterioration. Do not administer unstocked atropine/pralidoxime or use non-existent autoinjectors under this protocol.
Paramedic	All prior care plus advanced airway/ventilation, cardiac monitoring, and seizure or shock care under the applicable approved pathway. Coordinate agent-specific therapy with Medical Control and the receiving facility; no field antidote pathway is authorized.

Critical safeguards

- Do not enter a hot zone, vapor cloud, confined space, or oxygen-deficient atmosphere without required training, monitoring, PPE, and respiratory protection.

- Do not add external kit dosing or use the legacy North Carolina CHEMPACK, Mark I, or autoinjector instructions.

- Secondary contamination can injure responders and contaminate the ambulance or receiving facility; decontaminate before entry into the clean treatment area.

TE-08 | DECONTAMINATION, ESCALATION, AND HANDOFF

Scene safety and decontamination

Topic	Required action
Zones	Work through incident command and HazMat to establish hot, warm, and cold zones. Account for wind, terrain, vapor density, confined-space risk, flammability, and secondary-device concerns.
Clothing	Remove contaminated clothing and jewelry under appropriate protection; bag contaminated items and keep them out of the clean treatment area and ambulance.
Skin and eyes	Brush dry material before irrigation. Irrigate eyes/skin as directed by HazMat and Poison Help; avoid routine chemical neutralization or water use on an unidentified water-reactive material.
Supportive care	Suction, oxygenation, BVM ventilation, seizure care, shock care, cardiac monitoring, and early destination notification are the available Claiborne field priorities.

Antidote and operational boundary

Topic	Required action
No field kit	Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime field kit. This protocol does not authorize routine field autoinjector or antidote administration.
Future adoption	Atropine and pralidoxime are recognized antidotes for life-threatening cholinergic poisoning, but any Claiborne field pathway requires separately approved inventory, training, pharmacy/formulary review, and standing orders.
Consultation	Call Poison Help and Medical Control early. Coordinate with HazMat/mutual aid and the receiving facility for agent identification, specialty resources, destination, and any care outside standing orders.

Handoff and linked pathways

- Report agent/source, route, duration, scene zone, required PPE, symptoms, decontamination, airway/ventilatory support, glucose/ECG findings, treatments, and Poison Help/Medical Control guidance.

- Use TE-09 Chemical / HazMat Exposure for general zone, PPE, decontamination, and agent-card guidance; use TE-07 for overdose/toxic-ingestion support when applicable.

- This direct source PDF replaces the prior imported sheet and matches Claiborne-approved TE-08 app content reconciled October 30, 2026.

Critical reminder: Supportive care, decontamination, responder safety, and early escalation are the approved Claiborne ground response. Do not delay these measures while seeking an unavailable field antidote kit.